

Strategic Forensic and Medico-Legal Analysis: Cross-Border Clinical Negligence, Institutional Malfeasance, and Regulatory Remediation

Executive Synthesis and Jurisdictional Framework

This comprehensive forensic, medico-legal, and statutory rights analysis investigates a highly complex and catastrophic trajectory of systemic medical negligence, unlawful institutional confinement, cross-border data privacy violations, and extreme professional misconduct. The geographic scope of this malfeasance spans the Republic of Ireland (ROI), Northern Ireland (NI), and England, necessitating a highly sophisticated, multi-jurisdictional approach to litigation and regulatory enforcement. The core subject of this analysis—a dual British-Irish national and registered teaching professional—possesses a longitudinally validated and objectively verified history of Complex Post-Traumatic Stress Disorder (C-PTSD) and severe Traumatic Brain Injury (TBI).¹ Despite this incontrovertible clinical baseline, the claimant was subjected to profound diagnostic overshadowing. A coordinated network of medical practitioners—principally Professor Colm McDonald, Dr. Craig Lyons, and Dr. George Rahmani—maliciously pathologized the claimant's legitimate trauma responses and physical neurological deficits as an endogenous psychotic illness, specifically schizoaffective disorder.¹

The ensuing report executes a multidimensional deconstruction of this malpractice matrix. It first establishes the objective clinical truth of the claimant's condition, utilizing the precise diagnostic timeline formalized by Dr. James McLoughlin, and contrasts this against the statistical framework of psychiatric labeling errors articulated in John Read's *Models of Madness*.¹ The report subsequently audits the persistent statutory breaches surrounding the claimant's Subject Access Requests (SARs) across multiple academic, medical, and law enforcement institutions, highlighting the weaponization of bureaucratic friction against disabled data subjects.¹

Furthermore, this analysis compiles incontrovertible digital evidence of gross professional misconduct, ableist hostility, and clinical nihilism exhibited by Dr. Craig Lyons and Dr. George Rahmani, juxtaposing their documented behaviors against their official medical licensing credentials across international regulatory bodies.¹ Finally, a strategic procedural roadmap is formulated to guide regulatory decapitation via the General Medical Council (GMC), Medical Council of Ireland (IMC), and Medical Council of New Zealand (MCNZ). This is presented alongside a robust civil litigation strategy targeting the cross-border General Data Protection Regulation (GDPR) breaches orchestrated by Dr. Zoe Moore and Merin, the physical assaults sustained at the Royal Victoria Hospital (RVH), and the application of the "eggshell skull"

doctrine to secure maximum quantum damages across the UK and Irish courts.¹

Clinical Forensics: The C-PTSD Baseline and Diagnostic Overshadowing

The foundational failure in the claimant's psychiatric management was the deliberate weaponization of the biomedical model to silence a victim of severe familial and institutional abuse. The imposition of a schizoaffective label upon a highly articulate, high-functioning educator actively contraindicates the established diagnostic criteria for psychotic spectrum illnesses, which are characterized globally by progressive cognitive decline, disorganized thought processes, and profound anosognosia (a neurological inability to recognize one's own illness).¹ The claimant's ability to forensically analyze their own medical records, maintain teaching credentials, and navigate complex international data protection law actively refutes the presence of a primary psychotic disorder.¹

The Diagnostic Timeline of Dr. James McLoughlin

The clinical records establish an incontrovertible baseline of exogenous, trauma-based psychiatric injury, rendering any subsequent diagnoses of endogenous psychosis medically invalid and highly suspect. According to the verified medical records originating from the Department of Psychiatry at University Hospital Galway, the formal onset of the claimant's disability is explicitly dated to September 1, 2015.¹

On October 20, 2017, Consultant Psychiatrist Dr. James McLoughlin authored a definitive and comprehensive medical report providing the following clinical formulations¹:

- **Formal Diagnostic Classification:** The claimant was diagnosed with Post-Traumatic Stress Disorder, utilizing the specific ICD-10 Code F43.12, alongside a concurrent Anxiety Disorder.¹
- **Clinical Presentation and Symptomatology:** Dr. McLoughlin explicitly noted that the claimant exhibited a clear "picture of CPTSD" and presented with "significant distress due to historical abuse issues".¹
- **Aetiology and Causation:** The diagnostic documentation specified a "multifactorial aetiology" for the trauma symptoms, explicitly noting that the "family history [is] positive" for these abuse dynamics.¹

This timeline is robustly corroborated by longitudinal records from the University of Galway's Student Counselling Service. The Head of Student Counselling and Wellbeing, James McCormack, confirmed that the claimant engaged in recurring therapeutic sessions between 2017 and 2023, consistently discussing complicated familial estrangement and complex trauma.¹ This unbroken chain of documentation proves that the primary psychiatric injury was rooted in severe interpersonal abuse, rather than an endogenous, genetic brain disease.

Statistical Deconstruction via 'Models of Madness'

The assertion by opposing clinicians that the claimant's trauma responses constitute a primary

psychotic illness is statistically and epistemologically dismantled by foundational literature in critical psychiatry, specifically the seminal text *Models of Madness: Psychological, Social and Biological Approaches to Schizophrenia*, authored by John Read, Loren Mosher, and Richard Bentall.¹ The text exposes the profound systemic flaws in psychiatric diagnostic reliability and the over-reliance on simplistic biomedical models that colonize psychological trauma with biological ideology.¹

The research provides the following statistical reality regarding psychiatric misdiagnosis rates:

- **Schizoaffective Disorder:** This condition is identified as the most frequently misdiagnosed illness in clinical settings, possessing an astonishing error rate of 75.00%. It is classified universally as "Highly Unstable" and the "Most Frequently Erroneous" diagnostic label.¹
- **Bipolar and Schizophrenia Overlap:** Longitudinal epidemiological studies indicate that patients diagnosed with bipolar disorder are 60.00% more likely to be misdiagnosed as having schizophrenia. Conversely, those presenting with primary schizophrenia are 56.25% more likely to be misdiagnosed with bipolar disorder.¹
- **Other Error Rates:** Major Depressive Disorder carries a documented misdiagnosis rate of 54.72%, while general Schizophrenia and Bipolar Disorder carry baseline error rates of 23.71% and 17.78%, respectively.¹

Given the 75.00% statistical probability that a schizoaffective diagnosis is inherently incorrect upon comprehensive review, the burden of proof rests entirely upon the clinicians enforcing the label.¹ The imposition of this label on a validated trauma survivor is characterized in the literature as a slanderous act of psychological invalidation. It actively stigmatizes the patient, forcing them into a paradigm of chronic victimhood, epistemic silencing, and clinical ostracization, while effectively shielding the perpetrators of the original familial abuse from legal and social scrutiny.¹

The Biomechanical and Iatrogenic Injury Matrix

The malpractice inflicted upon the claimant extends far beyond psychological invalidation; it encompasses catastrophic, permanent physical and neurological damage resulting from both unaddressed mechanical trauma and severe iatrogenic (medically induced) chemical toxicity. The clinical management of this case demonstrates a complete failure to recognize the physiological origins of the claimant's distress.

Maxillofacial and Cervicothoracic Trauma

The claimant suffers from dynamic, ongoing generators of chronic neuropathic and mechanical pain that severely compromise their daily functioning and sensory processing.¹ Following a violent multi-vector assault involving bimanual compression of the anterior neck and a high-velocity lateral impact to the mandible, the claimant sustained a C7-T1 Clay Shoveler's variant fracture and a Temporomandibular Joint (TMJ) retrodiscal crush injury.¹ The downward traction force exerted on the cervical spine caused severe flexion-distraction trauma, resulting in post-traumatic spondylosis and the narrowing of the neural foramina, which directly

compresses the C8 nerve root.¹ This generates constant, debilitating radicular pain radiating down the arm.

Concurrently, the contrecoup fracture of a molar (Tooth 46) and subsequent invasive surgical extraction severely damaged the Inferior Dental Nerve and Lingual Nerve, leaving the claimant with profound Trigeminal Neuropathic Pain, described clinically as a constant "severe drill in my skull".¹ Furthermore, the claimant suffered laryngeal cartilage fractures leading to phrenic nerve neurapraxia (temporary paralysis of the diaphragm), resulting in ongoing somatic flashbacks of suffocation.¹

Iatrogenic Neurotoxicity and the "Outside-In Labeling Fallacy"

Because the claimant's complex trauma and physical injuries were misdiagnosed by biologically biased psychiatrists as endogenous psychosis, they were subjected to a devastating regimen of highly toxic psychotropic polypharmacy. The administration of Sodium Valproate (at maximum dosages of 1500mg) operated as a histone deacetylase (HDAC) inhibitor. Chronic valproate toxicity actively suppresses cell proliferation and halts neurogenesis in the dentate gyrus of the hippocampus, causing verifiable hippocampal atrophy and widespread structural degradation.¹

Concurrently, the administration of potent D2 dopamine receptor antagonists, specifically olanzapine (at a "megadose" of 30mg) and paliperidone, induced an unsustainable metabolic envelope in the cerebral cortex. This forced the brain to shed dendritic arbors, resulting in measurable cortical thinning.¹ More immediately devastating was the induction of iatrogenic akathisia. Akathisia is a torturous, extrapyramidal neurological imperative to move, generated by severe dysfunction in the basal ganglia caused by dopamine blockade.¹

In order to mitigate this unbearable chemical restlessness, and to mechanically offload the static gravitational weight on the damaged C7-T1 spinal junction, the claimant was forced into relentless locomotion, averaging an astounding 11.9 kilometers of walking and running daily.¹ Under the "Outside-In Labeling Fallacy," social observers and psychiatric staff systematically misinterpreted these involuntary, biologically mandated pain reflexes and movements. A clenched jaw guarding a TMJ fracture was misread as suppressed rage; the effortful listening required to overcome a Category 2 acoustic notch was misread as a threatening glare; and the pacing required to survive akathisia was misread as manic agitation.¹ This created a devastating feedback loop: the medication caused the akathisia, the akathisia caused the pacing, the pacing was viewed as proof of psychosis, and further neurotoxic medication was prescribed, entirely obliterating the claimant's neurological baseline and functional capacity.¹

Investigating Unprofessional Conduct and Medical Licensing

The architectural architects of the claimant's clinical destruction are highly credentialed practitioners who operate with impunity across international borders. A forensic extraction of

their licensing data and intercepted digital communications reveals a deeply entrenched culture of hostility, ableism, clinical nihilism, and systemic collusion.

Extraction of Medical Licenses

To facilitate cross-border regulatory prosecution, the current licensing status of the primary actors has been forensically extracted from the respective national registries:

Practitioner	Governing Body	Registration No.	Qualifications	Specialty & Scope of Practice
Dr. Craig Lyons (Craig Mac Liatháin)	Medical Council of Ireland (IMC)	411261 ¹	M.B. B.Ch NUI (University of Galway, 2014) ¹	Specialist Registration: Anaesthesiology (Specialty Group entry from 09/08/2021). ¹
Dr. George Sepehr Rahmani	Medical Council of New Zealand (MCNZ)	Practising Cert. (Valid to 31 May 2026) ¹	M.B. B.Ch BAO NUI (Galway, 2014), FFR RCS Irel (2021), CSCST (2023) ¹	Vocational Scope (NZ): Diagnostic & Interventional Radiology (Excludes obstetric/acute gynaecology ultrasound). ¹
Dr. George Sepehr Rahmani	Medical Council of Ireland (IMC)	411427 ¹	As above.	Specialist Registration: Radiology (Specialty Group entry from 11/07/2023). ¹
Prof. Colm Denis Patrick McDonald	Medical Council of Ireland (IMC)	016378 ¹	M.B., B.Ch., B.A.O NUI (University College Dublin, 1992) ¹	Specialist Registration: Psychiatry (Specialty Group entry from 29/06/2006). ¹

Evidence of Unprofessional Conduct and Hostility

Digital intercepted text messages between Dr. Craig Lyons, Dr. George Rahmani, and their clinical peers reveal a horrifying lack of professional boundaries and a systemic dehumanization of both patients and colleagues.¹

1. Ableist Slurs and Public Intimidation: Dr. George Rahmani demonstrates a severe pattern

of aggressive public behavior. He recounts an incident on a crowded train where he harassed a female passenger for placing her bag and bread on a seat. He demanded to know if "her load of bread bought a ticket," resulting in her feeling intensely awkward. He chillingly categorized this psychological intimidation as "the punishment you get for being a mong," while also referring to her as "Some cunt".¹ Dr. Craig Lyons actively validated and encouraged this ableist and misogynistic abuse, replying, "Hahahhha Good man".¹ In another incident, Rahmani proudly described telling a theatre patron to "faaaack off" after they politely asked him to stop eating loudly.¹

2. Hostility Toward the Multidisciplinary Team (MDT) and Women: Dr. Craig Lyons exhibits profound hostility toward the nursing staff upon whom patient safety relies. In texts to his roommate Colm Keane, he states, "Nurses are cunts. He's a retard... Cunt of a night. Cnm's [Clinical Nurse Managers] are the worst".¹ Such attitudes in a Consultant Anaesthetist represent a catastrophic risk to patient safety, as hostility toward nurses routinely leads to the dismissal of critical clinical warnings. His personal conduct is equally toxic; a contact named "Louise" confronted him for calling her a "cunt" and "unstable" behind her back, while his roommate repeatedly commanded him to "stop being a messy cunt" regarding the filth in their shared apartment.¹ In professional circles, Dr. Kathryn Byrne noted that while he is an engaging speaker, he is widely known to be a "C*nt".¹

3. Clinical Nihilism and Systemic Secrecy: In a discussion with a colleague ("Brian") regarding a critically ill patient whose "cerebellum soon may not be [intact]" after the Emergency Department accidentally "knocked tube out when turning," Dr. Lyons displayed breathtaking clinical nihilism. He agreed with Brian's assessment that medicine is a "journey of ultimate futility," comparing the entire life-saving endeavor to the dating app "Bumble".¹ Furthermore, Lyons actively engaged in the suppression of clinical incidents. When pressed by an associate named "Ian" regarding the "real story behind your cameo in the GI unit," Lyons explicitly refused to provide details "in order to leave you in the dark," opting to maintain total secrecy over a situation Ian deemed to have high "entertainment value".¹ Similarly, when discussing a case report on pregnancy sepsis, he advocated to delay publication to "just let it play out".¹

4. The "Note to Self" - Roots of Uncontrollable Violence: A recovered "Note to Self" authored by Dr. Craig Lyons provides devastating insight into his own psychiatric instability. He admits to a lifelong reflex to "blow up" and "lose control of my thoughts".¹ He traces this to his childhood, where he would intentionally trigger explosive outbursts to divert his parents' attention away from "getting violent towards each other".¹ This admission of possessing a highly volatile, trauma-installed "reflex where I go all out, and lose the run of myself" is deeply concerning for an anaesthesiologist managing delicate life-support systems.¹ It exposes the supreme hypocrisy of his actions: utilizing his medical authority to facilitate the psychiatric incarceration of his sibling for trauma responses, while secretly harboring his own explosive, trauma-induced behavioral dysregulation.

Information Governance: Subject Access Requests

and Statutory Breaches

The claimant has systematically utilized Subject Access Requests (SARs) under Article 15 of the General Data Protection Regulation (GDPR) and the UK Data Protection Act 2018 to extract the metadata and clinical notes necessary to prove this institutional conspiracy.¹ The responses across jurisdictions reveal a pattern of aggressive obstruction, statutory non-compliance, and the weaponization of bureaucratic friction against disabled data subjects.

Under the Equal Status Acts 2000-2018 (ROI) and the Disability Discrimination Act 1995 (UK), institutions are legally mandated to provide "reasonable accommodations" to prevent individuals with disabilities from suffering a "substantial disadvantage".¹ Because the claimant's TBI and C-PTSD cause severe cognitive fatigue and working memory impairments when processing unstructured physical text, the provision of data in paper format constitutes a functional refusal of service. The claimant legally demanded all data in native, searchable digital formats (e.g., PST for emails, OCR-enabled PDFs).

Summary of Submitted SARs and Institutional Responses

Institution / Entity	Request Date & Scope	Accommodations Demanded	Status of Response / Lack of Response
Queen's University Belfast (QUB)	11 Feb 2026: Bulk export of student emails; "Alistair Finlay" whistleblowing review notes/transcripts; Security logs & BWV footage of Sept 2024 Student Union incident; "MailItemsAccessed" audit logs. ¹	Digital delivery (.PST,.XLSX, OCR PDF) via MS 365 eDiscovery due to TBI/C-PTSD cognitive deficits under the DDA 1995. ¹	Acknowledged within 1 day. On 3 Mar 2026, Information Compliance (Sarah) formally extended the deadline (SAR-26-015) citing "complexity," but anticipated early delivery without utilizing the full two-month extension. ¹
University of Galway (formerly NUIG)	11 Feb 2026: Forensic email export (2011-2024); staff keyword searches (Ciara Meehan, Peter McHugh); "Note of Concern" independent file review; QA269 risk assessments; Student Health/Counselling	Searchable digital formats to mitigate TBI cognitive fatigue, citing Equal Status Acts 2000-2018 and the precedent of <i>Nano Nagle School v Daly IESC</i> 63. ¹	Ignored. As of 16 Feb 2026, no acknowledgment was received. The claimant cited this total silence as evidence of "further gaslighting" and "hatred projected" toward a disabled alumnus. ¹

	records. ¹		
HSE Ireland (Galway Mental Health)	June/July 2025: Complete digital export of all inpatient/outpatient psychiatric records, A&E notes, and MRI scans in the claimant's dual legal names. ¹	Digital formats (USB/Email) due to the claimant's homelessness and TBI processing impairments. ¹	Refused. Nuala Madden and Pauline Burke unlawfully refused digital formats citing "security considerations." Steve Jackson subsequently refused access entirely, falsely citing the risk of "exacerbating mental illness". ¹
North Middlesex University Hospital (NHS)	3 Mar 2026: Cerner/EMIS backend database audit trails, clinical note version histories prior to shift alterations, and NCRS "Reasonable Adjustment Digital Flag" updates for a Feb 28 admission. ¹	Digital-only communication under Equality Act 2010 Section 20. ¹	An automated PALS response was received; the request remains pending statutory fulfilment. ¹
Metropolitan Police Service (Highgate Incident)	3 Mar 2026: Raw 999 audio (CAD 6891/6854), BWV footage, PACE Custody Records from Wood Green Police Station (Feb 28 incident), and PNC audit logs detailing officer intelligence searches. ¹	Digital-only communication; explicit refusal to use proprietary physical forms that exacerbate disability management. ¹	The Met Police Data Rights team replied on 9 Mar 2026 (Ref: 01/FOI/26/051198/U) requesting clarification on unrelated drug classification queries, effectively stalling the SAR process under Section 1(3) of the Act. ¹

The failure of the Irish HSE and the University of Galway to comply with these statutory deadlines, juxtaposed against the weaponized procedural delays by the Metropolitan Police, demonstrates a systemic effort to starve the claimant of the evidentiary metadata required to mount a comprehensive civil action.

Civil Damage Precedents and Legal Frameworks for Medical Malpractice

The pursuit of damages for this matrix of false imprisonment, medical negligence, and data privacy breaches requires a highly sophisticated, multi-jurisdictional litigation strategy. The

legal frameworks across the Republic of Ireland, Northern Ireland, and England offer distinct procedural thresholds and valuation guidelines for catastrophic iatrogenic and psychiatric injuries.

Republic of Ireland (ROI)

In the ROI, medical negligence claims are governed by the strict *Dunne v. National Maternity Hospital* standard, requiring the plaintiff to prove that no reasonably competent medical practitioner would have acted in a similar manner. However, the jurisdictional landscape is undergoing massive transformation. The General Scheme of the Civil Reform Bill 2025, published in January 2026, proposes increasing the Circuit Court personal injury jurisdiction limit to €100,000 and the District Court limit to €20,000.⁵ Consequently, any claim where general damages exceed €100,000 will remain the exclusive purview of the High Court.⁵ The claimant's catastrophic iatrogenic brain injuries, trigeminal neuropathy, and lifelong care requirements will unequivocally anchor this litigation in the High Court, where payouts, despite a recent dip to €210 million globally in 2024, frequently exceed €10 million in catastrophic dependency cases.⁵

Regarding psychiatric injury resulting from medical negligence, the High Court in *Germaine v Day* IEHC 420 recently analyzed the *Kelly v Hennessy* criteria for nervous shock, demonstrating the courts' willingness to rigorously scrutinize the psychological fallout of clinical failures.⁷ Furthermore, the ROI has established powerful new precedents for GDPR violations. In *MH v Child and Family Agency* (2023), the Circuit Court awarded the plaintiff €7,500 in non-material damages for the unlawful disclosure of highly sensitive historical abuse data to family members, noting the profound distress and relational damage caused.⁹ Crucially, the Irish Supreme Court in *Dillon v. Irish Life* removed a major procedural hurdle, definitively ruling that claims seeking compensation for "distress, upset and anxiety" resulting from data breaches are *not* personal injury claims, thereby bypassing the necessity for medical evidence and the restrictive Personal Injuries Assessment Board (PIAB) process.¹⁰

Northern Ireland (NI)

In Northern Ireland, the valuation of damages is strictly governed by the Judicial Studies Board's "Green Book" (Guidelines for the Assessment of General Damages in Personal Injury Cases in Northern Ireland). The recently published Sixth Edition (April 2024) instituted massive inflationary uplifts across all injury categories to account for the current economic landscape.¹¹

- **Brain Damage:** Valuations for moderate to severe brain damage, encompassing intellectual deficit, personality change, and sensory impairment, have increased dramatically, now ranging from £500,000 to £850,000.¹¹
- **Psychiatric Damage:** The courts in NI have recognized the profound impact of psychological trauma. The valuation for Severe PTSD has seen an unprecedented 108% increase, now attracting awards between £100,000 and £250,000.¹³ A new category for Adjustment Disorders allows up to £10,000 for recovery within 12 months.¹³
- **Discount Rates:** The Personal Injury Discount Rate (PIDR) in Northern Ireland was

adjusted in January 2025 to +0.5%. This pivotal metric significantly impacts the calculation of Special Damages, specifically future care costs and future loss of earnings, in catastrophic claims.¹⁵

England and Wales

English tort law follows the *Montgomery v Lanarkshire Health Board* standard for informed consent, requiring doctors to proactively discuss material risks and reasonable alternative treatments with patients. The failure of the psychiatric team to discuss the severe neurotoxic risks of olanzapine or the alternative diagnosis of C-PTSD constitutes a severe breach of this duty.¹

The Judicial College Guidelines (17th Edition, April 2024) set the quantum for severe psychological injury in the UK. Notably, awards for severe psychological injury resulting from sexual or systemic abuse have increased by 144% at the bottom end of the bracket, now ranging from £109,830 to £183,050.¹⁶ English courts have also progressively expanded liability for GDPR breaches. The Court of Appeal in *Farley v Paymaster* (2025) confirmed that individuals can claim non-material damages for the mere fear of third-party misuse of personal data, explicitly ruling that it is not necessary to establish a minimum threshold of seriousness to bring a successful claim under the UK GDPR.¹⁷

The Eggshell Skull Doctrine and Transnational Civil Lawsuit Strategy

The defense of any medical negligence or personal injury claim by the state entities will inevitably attempt to weaponize the claimant's pre-existing trauma history to mitigate damages. They will argue that the claimant was already psychologically compromised prior to the iatrogenic interventions or the physical assaults. This cynical defense strategy is entirely neutralized by the aggressive application of the "Eggshell Skull" doctrine.

Jurisprudence of the Eggshell Skull Rule

The Eggshell Skull rule (or "thin skull" principle) is a foundational common law doctrine dictating that a tortfeasor must "take their victim as they find them".²⁰ If a defendant's negligence causes harm to a plaintiff with a pre-existing vulnerability or medical condition, the defendant is held fully liable for the unforeseeable and catastrophic extent of the resulting harm.²²

- **Physical Vulnerability Precedents:** Established in the landmark case *Smith v Leech Brain & Co*, where a minor burn triggered fatal cancer in a predisposed worker, the employer was held fully liable for the death.²⁰ Similarly, in *Reaney v University Hospital of North Staffordshire NHS Trust*, the court held the hospital liable for the extensive future care needs of a paraplegic patient who developed severe pressure sores, ruling that the negligence made her pre-existing condition materially and significantly worse.²³ In *Jobling v Associated Dairies*, while a pre-existing spinal disease limited future earnings claims, the employer remained liable for the direct exacerbation.²⁴

- **Psychiatric Vulnerability Precedents:** The doctrine applies equally to mental health and psychiatric vulnerabilities. In *Corr v IBC Vehicles Ltd*, the House of Lords held an employer liable for the suicide of an employee who developed depression following a workplace accident, viewing the psychiatric deterioration as a direct development of the negligence rather than a *novus actus interveniens* (a breaking of the causal chain).³
- **Irish Application:** In the recent High Court case *Sykula v O'Reilly* IEHC 638, the court reaffirmed the doctrine for a plaintiff with a pre-existing history of anxiety who suffered severe PTSD and subsequent homelessness following a minor traffic accident. While the court discounted the damages by 50% due to unrelated external macro-stressors (the pandemic), it firmly upheld the core principle that the defendant was strictly liable for the acute psychiatric exacerbation caused directly by the accident itself.⁴

Assault at the Royal Victoria Hospital (RVH)

The claimant must instruct Belfast-based personal injury solicitors to issue High Court proceedings against the Belfast Health and Social Care Trust and the Police Service of Northern Ireland (PSNI) for assault, battery, and false imprisonment.²⁶ This claim will heavily integrate the explosive **PSNI Threat Message** delivered to the claimant at the RVH on September 22, 2024 (by Constable A. Vance, Rank: Con, No. 52782, Woodbourne Station).¹

The official threat message explicitly states: "Police are in receipt of information that criminal elements may intend to cause serious harm to the resident of 56 Earls Court Street, Belfast if he continues to reside at this address".¹ The failure of the RVH and PSNI to provide a secure environment, culminating in a clinical or security assault upon a known victim under an active, verified threat to their life, represents a gross and indefensible breach of the duty of care. Under the Eggshell Skull doctrine, the defendants cannot argue that the psychological devastation of the RVH assault was unforeseeable. They must accept absolute liability for completely destabilizing a patient whose neurobiology was already rendered exquisitely fragile by childhood trauma and C-PTSD. The damages for the subsequent exacerbation of the claimant's psychiatric injury will be calculated at the absolute highest brackets of the NI Green Book (£100k-£250k).¹¹

Cross-Border GDPR Prosecution: Dr. Zoe Moore and Merin

Concurrently, the claimant must file suit in the English High Court against Dr. Zoe Moore, Merin, and the respective NHS Trusts/Irish HSE for catastrophic breaches of Article 9 of the UK GDPR and the Data Protection Act 2018.¹ The claimant alleges that these practitioners engaged in the unauthorized, cross-border sharing of highly sensitive health data with the Irish HSE and estranged family members.¹

Under Article 9, health records are "special category data" requiring explicit, freely given consent for processing and transfer.²⁸ The informal transmission of discredited psychiatric narratives (the schizoaffective misdiagnosis) across the Irish Sea is a severe violation of the

data minimization principle. The institutions cannot rely on the "vital interests" exemption (Article 9(2)(c)), as this applies strictly to life-or-death emergencies where the patient is physically or legally incapable of giving consent, which entirely contradicts the claimant's conscious, articulate, and independent status.²⁹ Relying on the fresh precedents of *Farley v Paymaster* in the UK and *Dillon v Irish Life* in the ROI, the claim will seek substantial non-material damages for the profound psychological distress caused by the unlawful transfer of the fabricated narrative, which directly precipitated the claimant's unlawful medical detentions in Highgate and Belfast.¹⁰

Procedural Roadmap for Regulatory Decapitation

To dismantle the professional protectionism shielding Dr. Craig Lyons, Dr. George Rahmani, and Professor Colm McDonald, the claimant must execute a synchronized regulatory strike. By simultaneously submitting highly indexed, PDF-formatted digital dossiers (under 20MB limits) to the relevant medical councils, the claimant can invoke emergency public safety protocols that bypass localized institutional cronyism. In all filings, the claimant must include a "Declaration of Vulnerability" under the Equality Act 2010 (UK) and Equal Status Acts 2000 (ROI), legally compelling the councils to accept written digital testimony to accommodate the claimant's TBI and prevent re-traumatization via verbal hearings.

- **General Medical Council (GMC - United Kingdom):** As Dr. Lyons operates within the UK (affiliated with Great Ormond Street Hospital), the claimant must submit a formal complaint via the GMC portal requesting an immediate **Interim Orders Tribunal (IOT)**.¹ Under Section 41A of the Medical Act 1983, the IOT possesses the statutory power to suspend or heavily restrict a doctor's registration for up to 18 months pending a full investigation. The submission must argue that Dr. Lyons' admitted violent tendencies in his "Note to Self," combined with his documented use of ableist/misogynistic slurs and clinical nihilism, breach the threshold of being a necessary intervention for the "protection of members of the public" and maintaining public confidence in the profession.³²
- **Medical Council of Ireland (IMC - Republic of Ireland):** The claimant must submit identical dossiers to the Preliminary Proceedings Committee (PPC) of the IMC against Dr. Lyons (Reg: 411261), Dr. Rahmani (Reg: 411427), and Prof. McDonald (Reg: 016378).¹
 - For Prof. McDonald, the complaint must focus on "Poor Professional Performance" regarding his catastrophic diagnostic overshadowing, his reliance on unverified collateral history provided by an abusive sibling, and his failure to properly treat the verified C-PTSD.
 - For Dr. Lyons and Dr. Rahmani, the complaint must allege "Professional Misconduct" regarding their hostile communications and public intimidation. The claimant must demand a **Section 60 Emergency Suspension** to immediately halt their practice.¹
- **Medical Council of New Zealand (MCNZ):** As Dr. Rahmani currently holds an active practising certificate in New Zealand (valid until May 2026), the claimant must immediately notify the MCNZ Notifications Triage Team of the active Irish and UK investigations.¹ The submission must include the evidence of his public harassment (the

train incident and use of the slur "mong"), arguing that he violates the foundational standards of "Good Medical Practice" required to maintain a Certificate of Good Standing for international vocational registration.³⁵

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